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June 16, 2025

The Honorable Mehmet Oz
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U.S. Department of Health and Human Services
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Thomas Keane
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National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C Street, SW
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RE: WellSpan Health's Response to *Request for Information; Health Technology Ecosystem* (CMS-0042-NC)

WellSpan Health (WellSpan) is pleased to provide comments to the Centers for Medicare & Medicaid Services (CMS) and the Office of the National Coordinator for Health Information Technology (ONC) regarding the market for digital health products for Medicare beneficiaries as well as the state of data interoperability and broader health technology infrastructure.

WellSpan's vision is to reimagine healthcare through the delivery of comprehensive health and wellness solutions throughout our continuum of care. As an integrated delivery system focused on leading in value-based care, we encompass more than 3,000 aligned physicians and advanced practice providers, more than 250 locations, nine award-winning hospitals, home care, and a behavioral health organization serving central Pennsylvania and northern Maryland. Our high-performing Medicare Accountable Care Organization (ACO) is the region's largest and one of the best in the nation. We serve nearly 900,000 unique lives, with 274,000 under value-based contracts and 50,000 in our ACO.

To improve the state of data interoperability in the health technology ecosystem, WellSpan recommends the agency focus on the following areas:

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- Ensuring Patient Access
  - We can aim to incentivize patient health behaviors and engagement;
  - We can enhance developer access through Blue Button and enable more innovation;
  - We can empower patients and market competition through enhanced provider benchmarking;
  - We can continue to expand access to virtual care in primary and specialty care; and
  - We can continue to improve and increase the use of care at home technology.
- Securing Provider Access to Data
  - We can improve patient care and management through mitigating information asymmetry; and
  - We can improve program design and accountability through more accessible claims across the health ecosystem.
- Payer-to-Payer Enhancements
  - We can improve payer care management and provider access to longitudinal claims data despite member churn and migration patterns.
- Improving Prior Authorization
  - We can reduce administrative waste and patient delays by eliminating latency and enhancing accuracy of prior authorization processes.

PC-1.

What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

We appreciate CMS's interest in driving digital innovation to improve the health of Americans and encourage CMS to pursue interoperability of health management and care navigation mobile applications to allow for better care coordination by physicians and care teams. WellSpan's patients rely on our patient portal, with 72.9% of them active within the last 18 months. Parents are particularly engaged - with 84.6% of pediatric patient proxies active on the portal. Outside the portal, some of our more active patients use health tracking apps connected to wearables, such as Apple Health with Apple Watch, FitBit's devices and its app, Garmin Connect with Garmin smartwatches, and others. These patients tend to use the apps for personal goals and wellness rather than for chronic disease management. Patients living with chronic diseases are more likely to use specialized devices, such as blood pressure cuffs, glucometers, and pulse oximeters.

At WellSpan, we are pioneering the use of portal tools to advance community health. We have 14 educational programs ("Care Companions") that help patients navigate through episodes of care or dealing with chronic conditions. We have also used automated messaging combined with self-triage tools to improve completion of regular health maintenance topics. We have increased bone density scans, use of Fecal Immunochemical Test (FIT) kits for colorectal cancer screening, and kidney health testing using these tools.

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PC-1a.

a. ***What are the top things you would like to be able to do for you or your loved ones' health that can be enabled by digital health products?***

Based upon our patients' behaviors, we would prioritize viewing test results, messaging with their care team, scheduling appointments, pre- and post-care education, health monitoring, timely and meaningful notifications, virtual visits, and completing self-assessments.

These last three functions are critical: the first ensures active monitoring for at-risk patients while they are at home, the second greatly increases access for patients, and the third promotes active health management outside of provider visits.

PC-2.

Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

Patients who rely on a single health care provider generally have easier access to their health information in one place. However, many patients rely on multiple health care providers and maintain multiple patient portal accounts. Some of these portal accounts can be linked together to provide easier access to health information through proprietary electronic health record (EHR) system capabilities like our own Epic MyChart. Four of the five largest health care systems in central Pennsylvania use Epic MyChart.

Having health information in one place is especially important for patients managing complex care journeys like cancer care or multiple chronic conditions, as we often see with our Medicare population. Patients may be seeing an oncologist, lab and imaging provider, and primary care provider with three different health record systems. This makes it extremely complex to track appointments, prescriptions, and care management across various portals. Having health information in one place is also important for patients to stay on top of preventive health tasks. For example, patients may receive mammograms or other important screenings at providers not affiliated with their primary care provider.

We note that CMS's *Interoperability and Patient Access Final Rule* (85 FR 25510) requires Medicare Advantage (MA) organizations, Medicaid Fee-for-Service (FFS) Programs, Medicaid managed care plans, Children's Health Insurance Program (CHIP) FFS programs, CHIP managed care entities, and Qualified Health Plan issuers on Federally-Facilitated Exchanges (FFEs) to implement application programming interface (API) technology to advance health data exchange. We believe these new regulations will advance consumers and physicians access to important patient data needed for modern risk stratification, care coordination, and care management, particularly the rule's new requirements for certain payers to

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implement three additional APIs: Provider Access API, Payer-to-Payer API, and Prior Authorization API.

PC-4.

What features are missing from apps you use or that you are aware of today?

Translating the highly clinical focus of a medical record to plain language is an important need. Test results and providers' notes are all appropriately clinically focused. But while we give patients greater access to clinical information, we must help them interpret what they are given. We regularly hear of patients reviewing test results on their own before discussing it with their provider, consulting the internet, and then terrifying themselves about what they are seeing. The same principle applies to providers' notes: things can seem scarier in the clinical language when there may not be anything to worry about.

Another development that would be challenging but highly desirable for patients is a way to help them prioritize across multiple care journeys. A patient with five tests for three conditions has little chance – outside of an office visit where ranking the tests is explicitly addressed with the provider – of knowing which is most important to schedule first.

And simply because it is important to the patient does not mean their local health system can accommodate them. Almost all health systems are struggling with this – when someone needs care, how do we get them to the most appropriate site of care? We deploy tools like online symptom checkers, but most of these are not taking the actual patient record into account. This is another place where the barriers EHR vendors put in place for innovation on top of the health record come into play. We are limited by the features and functionality that vendors release, and while many systems, including WellSpan, build their own solutions, the EHR companies are not incentivized to build APIs to integrate them.

PC-6.

What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

At WellSpan, we place a premium on good design, user experience (UX) best practices, and accessibility because we have found that Medicare beneficiaries are often eager to learn and use new technologies for their healthcare. Our MyWellSpan portal engagement scores – a composite measure of patients' likelihood to log on and manage their care – for patients aged 65-74 came within 5% points of younger users; this drops off more for patients over 75 years of age.

Older patients can be expected to have challenges with eyesight and manual dexterity, making access to health products on smartphones challenging. For these patients, easy pathways to ask questions as well as access to speech

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recognition technologies will help improve their use of digital health products. Critically – these technologies must directly integrate with the EHR to ensure accurate and timely transmission of the patient data and scheduling information. WellSpan is pioneering the use of an artificial intelligence (AI) assistant that can talk with patients on the phone in a conversational manner to promote completion of health maintenance topics, schedule appointments, and answer concerns about screenings.

PC-8.

In your experience, what health data is readily available and valuable to patients or their caregivers or both?

"Health data" in this context can be said to encompass three bodies of information:

- Patient-specific data, including Protected Health Information (PHI) concerning the patient's health and Social Determinants of Health;
- Data concerning provider/procedure availability, and;
- Data concerning insurance and the cost of care.

PHI is generally available to patients with portal accounts or active health tracking app usage. Test results and visit notes are readily available and of high value to patients. The biggest challenges for patient access are interoperability across providers. If their care involves using providers in different networks or health systems, getting them to communicate with each other can be extremely challenging. Further, the health information collected through standalone apps is often difficult to incorporate into the EHR, which poses challenges for WellSpan and others who need to aggregate PHI into meaningful cohorts for research purposes.

Provider/procedure availability data also varies widely by practice and practitioner. At WellSpan, we are committed to making access as simple and direct as possible, with patients able to schedule most routine and acute visits with their provider or someone in their provider's practice.

PC-8a.

a. ***What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?***

While highly valuable, the third category of pricing and claims is the thorniest for patients. Although payors are best positioned to provide this information, WellSpan does its best to provide pricing transparency, but our costs are translated for patients by their payors according to the terms of their insurance plans. The greatest barrier to transparency derives from the differences between each individualized plan, often meaning that no procedure will have a set cost. The best

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way to remove barriers to understanding cost and claims information is to ensure payor systems can communicate clearly with health providers and their patients.

PR-2.

What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

The biggest barrier is translating clinical experiences into discrete data. For example, capturing a foot exam in a way that satisfies quality measures currently requires the provider to document that exam in a specific way — different from how the provider likely documents the rest of the patient's exam. We do not want to force all providers to document certain ways because it impinges on their autonomy (and job satisfaction). Similarly, coding a visit (i.e., evaluation and management (E&M), modifiers, and additional codes) requires having certain elements documented in the encounter. Notwithstanding provider resistance, revenue and some quality measures are based on accurately submitting these codes. Nonetheless, we do not believe that we can trust thousands of providers to code accurately for all conditions.

This is where technology has an opportunity to make these tasks easier. The ambient AI documentation services have made strides in this direction and will continue to do so. Encouraging greater cooperation between EHRs and technology companies and health systems looking to innovate will remove barriers. The best innovations will result from a variety of approaches and solutions, not from uniformity. The adaptability of our platforms will be more important than having a single format for each part of the EHR.

PR-7.

What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

With the advent of cloud hosting and related technologies, flexibility between and across healthcare technology and Software as a Service (SaaS) vendors is increasingly affordable and practical. However, given lack of interoperability, deadweight loss associated “switching costs” results in an inefficient health information technology (IT) competitive market. In other words, competition is stifled because switching between technology platforms is disincentivized due to cost, complexity and data loss associated with such a transition. Improving inter-vendor interoperability requirements (recommend extending Trusted Exchange

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Framework and Common Agreement (TECFA) concepts beyond the initial Certified Electronic Health Record Technology (CEHRT) classes) would enable greater competition and innovation within the health IT vendor space. The benefit of a more efficient and competitive vendor landscapes is both direct and indirect improvements in patient cost and quality outcomes.

This comment also ties to our feedback in response to question PR-13, in which case civil monetary policies may be the best lever to consider for disincentivizing information blocking among Health IT vendors. We are aware of ongoing litigation in this space, and additional clarity from regulators would be helpful.

PR-8.

What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

Bulk exports to CMS would likely result in reduced provider administrative burden, particularly for smaller provider groups. At the same time, doing so would not force providers to develop the infrastructure and internal reporting needed to share and manage their own quality performance as a matter of standard operating and business practice.

We recommend establishing a patient/member threshold that allows direct bulk exports for smaller providers (e.g., independent practices), but excludes this option for larger providers so they are required to develop internal platform capabilities. This would allow for a balance between prioritizing accountability and usability. We recommend a standard dataset to simplify provider to vendor to CMS interoperability, but also recommend leaving it to the private market to compete on the efficiency of aggregating and reporting on quality measures to share back with CMS.

PR-13.

For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

With the advent of cloud hosting and related technologies, flexibility between and across healthcare technology and SaaS vendors is increasingly affordable and practical. However, given lack of interoperability, deadweight loss associated “switching costs” results in an inefficient health IT competitive market. In other words, competition is stifled because switching between technology platforms is disincentivized due to cost, complexity and data loss associated with such a transition. Improving inter-vendor interoperability requirements (recommend extending TECFA concepts beyond the initial CEHRT classes) would enable greater competition and innovation within the health IT vendor space. The benefit of a more efficient and competitive vendor landscapes is both direct & indirect improvements in patient cost and quality outcomes.

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Civil monetary policies may be the best lever to consider for disincentivizing information blocking among Health IT vendors. We are aware of ongoing litigation in this space, and additional clarity from regulators would be helpful.

VB-11.

What specific interoperability challenges have you encountered in implementing value-based care programs?

Enhanced analytics platforms, specifically those that integrate claims, clinical and ADTs, are now “table stakes” capabilities for running a successful value-based care (VBC) organization in 2025. The vendors that unlock these capabilities all have proprietary software, requiring significant start up direct investment costs and management opportunity-costs. These costs often are seen as barriers to entry for new VBC participants, and there are certainly barriers to switching between vendors. Neither of these situations is accretive to CMS’s stated goal of increasing VBC adoption by 2030.

WellSpan supports CMS’s value-based care goals, and we have led in value-based care, having saved the Medicare program over $80 million in costs in the last three years. Our recommendation here is focused on greater integration and interoperability with state data systems and Medicaid. Our nation is operating under an outdated system, which is an artifact from a pre-digital era whereby states administer their Medicaid programs on various data systems that do not integrate with each other - or with federal systems. In Pennsylvania, there are five Medicaid managed care regions each with four or five contracted Medicaid managed care organizations (MCO) and with carved out behavioral health MCOs each covering different geographies in one large complex patchwork quilt. The result is a disarray of data systems in use within the Medicaid program with huge costs to health systems to achieve interoperability with any one of them. Essentially, the health system must build integrated data sharing from the ground up in each instance of working with a different MCO data system. There are no standard definitions or data structures. Although Medicaid MCOs are required to share data with the state using a standard reporting protocol, health systems have no access to a standard data reporting approach throughout the Medicaid system. Given that Medicaid enrollees churn in and out of MCO enrollments, this has been a major barrier to true Medicaid value-based care innovation in Pennsylvania. The federal government should use its regulatory power to force greater inter- and intra- state integration of Medicaid data with the federal government’s federal Medicare dataset. This would enable massive innovation in Medicaid value-based care and care coordination for dual eligibles. It is a significant barrier for us which has prevented systems like WellSpan from taking on Medicaid risk.

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VB-12.

What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

We believe two key changes in CMS data management will accelerate adoption of value-based care models.

1. Real-Time quality feedback to ACOs. Medicare ACOs participating in the Medicare Shared Savings Program (MSSP) model currently receive insights into their quality performance only once annually via their year-end reconciliation. While some quality measures are sourced from clinical data, others are sourced from CMS data. Only sophisticated ACOs that have advanced data analytics and the ability to pay vendors can gain ongoing insights into quality performance. When ACOs receive their quarterly financial reports from CMS, the agency should also include quality data showing the ACO its progress in addressing quality measures. This would enable more timely action by ACOs to address quality opportunities for improvement and therefore allow them to improve quality and improve patient competition for higher quality care.

2. Greater integration with state data systems on Medicaid. Our nation is operating under an outdated system, which is an artifact from a pre-digital era whereby states administer their Medicaid programs on various data systems that do not integrate with each other or with federal systems. In Pennsylvania, there are five Medicaid managed care regions each with four or five contracted Medicaid MCO and with carved out behavioral health MCOs each covering different geographies in one large complex patchwork quilt. The result is a disarray of data systems in use within the Medicaid program. Although Medicaid MCOs are required to share data with the data using a standard reporting protocol, health systems have no access to a standard data reporting approach throughout the Medicaid system. Given that Medicaid enrollees churn in and out of MCO enrollments, this has been a major barrier to true Medicaid value-based care innovation in Pennsylvania.

VB-15.

How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

One of the most pernicious challenges facing healthcare is inter-organization communication. Phone calls and faxes are still prevalent. Even within a common infrastructure such as CCDs or Epic's CareEverywhere framework, data is still unstructured and inaccessible, severely limiting its practical utility in contemporary healthcare operations (not to mention the future of AI-enabled workflows). Fast

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Healthcare Interoperability Resources (FHIR) endpoints for each provider in the country would create the "mailbox" chassis needed to reform provider-to-provider communications. This could revolutionize care and disease management through improved coordination, polypharmacy mitigation, and removal of duplication and waste among other efficiency gains.

Just having the FHIR endpoints alone will not suffice. There needs to be additional structure and strategy incorporated. Providers are already overburdened with their EHR inboxes for refills, patient questions, peer communications, among other things. AI and other innovations are already addressing the EHR inbox conundrum, and those solutions are likely also applicable to inter-provider communications via a network of FHIR enabled network. But these AI solutions need a clean and consistently designed infrastructure to function best. Key data elements should include:

- Parent Organization Name and Taxpayer Identification Number (TIN);
- TIN — National Provider Identifier (NPI) relationships;
- Affiliated network(s) relationships;
- Practicing specialty; and
- An emergency escalation pathway/ phone number to urgently contact members of the provider's care team.

Conclusion

WellSpan believes these recommendations would greatly improve the data interoperability in the health technology landscape:

1. Simplify and provide technical support to convene stakeholders where dependencies exist.
2. Address market inefficiencies, specifically focused on the cost/complexity of data aggregation vendors. This includes reducing the barriers to entry and switching costs to encourage health IT market competition.
3. Set market expectations and structure by developing industry standard data definitions, file specifications and interoperability protocol that cross-cut payers, providers, patients and health IT vendors.
4. Preemptively address *Health Insurance Portability and Accountability Act* (HIPAA) related risk and tort reform requirements that will emerge in tandem with this next phase of data interoperability innovation.
5. Consider the economic concepts of patients' "willingness to pay" when it comes to tradeoffs between data privacy and expanded data integration/access. This framing should provide a more objective medium for comparing the (potentially significant) national health and efficiency gains downstream of a better integrated health IT ecosystem versus the more subjective and politically charged narratives associated with interpretations pursuant to HPIAA's Permitted Uses and Disclosures.

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Thank you for your consideration of these comments. WellSpan looks forward to serving as a resource as CMS to further our common goal of modernizing the nation's digital health ecosystem with a focus on empowering Medicare beneficiaries through greater access to innovative health technologies.

Sincerely,

DocuSigned by:
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WellSpan Health